

DECODED EXPLAINED

15th September, 2026

Gallant IAS Daily Current Affairs Initiative

UK unity 'threatened' by pact between Scotland, Wales, Northern Ireland

Associated Press

London, September 14

THE LEADERS of Wales, Scotland and Northern Ireland signed a pact Monday that threatens to lead to the breakup of the United Kingdom.

The first ministers of the three semiautonomous regions of the UK told PM Andy Burnham and his government to prepare for constitutional change and that their future lies with the European Union.

"For people watching across these islands — for people watching around the world — there could be no clearer sign that Westminster's time is coming to an end," they said in a memorandum of understanding, referring to Parliament.

The announcement came about 48 hours after US President Donald Trump waded into

the waters of the politically contentious issue when he said he would "love to see" a reunified Ireland during a trip to visit his golf course hosting Irish Open.

It's the first time the leaders of all three countries are either from pro-independence parties, or pro-unification in the case of Northern Ireland, which was partitioned and remained part of the UK when Ireland won independence from Britain over a century ago.

Burnham's spokesperson brushed aside the threat and said the prime minister strongly believes in the union and was more focused on lowering the cost of living than dealing with constitutional debates.

"The United Kingdom is at its best when people come together around our shared values and problem-solving, rather than division," Tom Wells said.



Hindi Diwas: How the Constituent Assembly resolved India's language question

Nikita Mohta

New Delhi, September 14

ON SEPTEMBER 14, 1949, after years of debate, the Constituent Assembly adopted what is commonly known as the Munshi-Ayyangar formula to settle the question of languages for official use in independent India.

Rather than declaring a 'national language', it designated Hindi as the 'official language of the Union', while allowing English to continue being used 'for all official purposes' for 15 years after the commencement of the Constitution. The date is now commemorated as Hindi Diwas, but the journey to achieving consensus was long, and language remained one of the most contentious issues before the Assembly.

Long-debated problem

At Independence, most Indians spoke one of roughly a dozen major regional languages, while Hindi was the most widely spoken language. English was the language of colonial administration, even though less than 1% of Indians spoke it.

The main questions then were whether English should be replaced by an indigenous language, and which language(s)

should have official status at the Centre.

Political scientist Hanna Lerner writes in *The Oxford Handbook of The Indian Constitution*, "The two main factions in the assembly debate over the national language were the representatives of the Hindi-speaking areas, mostly from north-central India, and the representatives of non-Hindi-speaking regions, particularly from the south, as well as the moderate leaders of the Congress Party."

The Hindi-speaking representatives demanded that Hindi be declared the national language and replace English immediately. Lerner writes, "They claimed that a multilingual society was incompatible with Indian unity. Seth Govind Das, a Congress representative of the Central Provinces and Berar, stated, 'We want one language and one script for the whole country. We do not want it to be said that there are two cultures here.'"

Representatives from non-Hindi-speaking regions rejected the idea that national unity required linguistic uniformity. Shri Shankarrao Deo of Bombay, the Congress General Secretary, stressed "Not uniformity but unity in diversity." Moderate Congress leaders, including Jawaharlal Nehru, recognised the practical difficulties



Language was one of the more contentious issues in front of the Constituent Assembly.

WIKIMEDIA COMMONS

of making Hindi the national language.

Majority or consensus?

The language dispute also raised a procedural question: how should a national language be chosen? Hindi proponents demanded a simple majority vote, while their opponents argued for consensus, preferably unanimity.

sus, preferably unanimity.

Seth Govind Das articulated the Hindi-speaking position: "We have accepted democracy and democracy can only function when majority opinion is honoured... Whatever decision is arrived at the majority must be accepted by the minority respectfully and without any bitterness."

By contrast, S P Mookerjee of Bengal warned against imposing a majoritarian decision on linguistic minorities: "If it is claimed by anyone that by passing an article in the Constitution of India one language is going to be accepted by all by a process of coercion, I say, Sir, that that will not be possible to achieve."

Rajendra Prasad, President of the Constituent Assembly, similarly argued, "Whatever decision is taken... will have to be carried out by the country as a whole... Even if we succeed in getting a particular proposition passed by majority, if it does not meet with the approval of any considerable section of people in the country... the implementation of the Constitution will become a most difficult problem."

A compromise is struck

The Assembly ultimately approved the Munshi-Ayyangar formula in 1950, later incorporated into Articles 343-351 in Part XVII of the Indian Constitution. It was named after K M Munshi and N Gopalaswamy Ayyangar, who were part of the Drafting Committee of the Constitution, with B R Ambedkar as its chairman. "Ultimately," notes Lerner, "it was the pragmatic consensus-seeking approach that triumphed."

The arrangement envisaged a 15-year transition, during which Hindi could increasingly be used by the Centre. Lerner adds, "In addition, the Constitution recognised fourteen other languages for official use (listed in the Eighth Schedule of the Constitution). The provincial governments were permitted to choose one of the regional languages or English for the conduct of their internal affairs, while English... remained the language of inter-provincial communication."

But when that period ended in 1965, Hindi was still not widely used by the government. Following violent riots in non-Hindi-speaking states in the 1960s, Parliament renounced the ideal of a national language. The Official Languages Act of 1963 later stated that English would continue to be upheld as an official language alongside Hindi.

"At first blush, since Article 343 designates Hindi as 'the official language of the Union', it would seem that the proponents of Hindi won," notes academic Sujit Choudhry in *The Oxford Handbook*, adding, "But upon closer examination the resulting set of constitutional provisions reflects a multidimensional compromise between these two positions."

Articles 343–351

Hindi in Devanagari script became the "official language of the Union"

"Official language" describes what the government uses for its business; "national language" claims something about collective identity

Official Languages Act, 1963

Munshi- Ayyangar formula

Experts back warnings for high level of each nutrient — not just fats, sugar, or salt content

They add that black and white designs are better as ‘they are harder to visually mask on colourful packaging’

Anonna Dutt
New Delhi, September 14

EVEN AS the Supreme Court is looking into a petition to implement mandatory front-of-pack labels on foods high in fats, sugar, or salt, global experts say such warnings should be triggered even if a food has high levels of a single nutrient. This is also what the country's food safety regulator, the Food Safety and Standards Authority of India (FSSAI), has proposed for the second phase of implementation of food labelling.

This view is also supported by the ICMR (Indian Council of Medical Research)-NIN (National Institute of Nutrition), whose Dietary Guidelines for Indians-2024 have been used as reference for deciding the nutrient thresholds for triggering the warning. The global experts are affiliated with organisations such as International Food Policy Research Institute, Harvard Medical School, Johns Hopkins University, National Institute of Public Health Mexico, University of Chile, University of

Sydney, among others.

These experts have appreciated the FSSAI's move to implement the red hexagonal warning label on the front of packaged food items, stating “warning labels are the only (type of) label that has real-world evidence of impact, including shifting consumer beliefs and behaviours, improving the nutritional profile of the food supply, and improving the healthfulness of purchases and dietary intake”.

They have given two important suggestions: One, the warning label should be triggered for each nutrient, meaning if a food is high in salt and fat, it should carry red hexagons stating so. Their statement said: “Multiple warning labels help signal overall product healthfulness: Evidence from Chile shows that consumers understand that products with more warning labels are less healthy than products with fewer or no warnings.”

Two, they have suggested that the warning use black hexagonal boxes instead of a red one, or mandate that the red



Experts have appreciated the FSSAI's move to implement the red hexagonal warning label on the front of packaged food items

ones are put on a white background. “A fixed, contrasting background prevents warnings from being visually absorbed into packaging: Colour-based labelling systems can also be less noticeable if they are placed over background packaging of a similar colour ... Without such a requirement, a coloured warning risks blending into similarly coloured packaging or being visually minimised through surrounding graphic elements, undermining the label's core purpose of rapid, at-a-glance identification.”

They added that black and white designs are better as “they are harder to visually mask on colourful packaging”.

A letter from the NIN director to the CEO of FSSAI has also made some important suggestions. The institute has recommended that the thresholds for identifying foods high in fats, sugars and salt be based on the total energy density of the food. “Therefore, the added fat and added-sugar cut-offs should ideally not be considered in isolation from the accompanying energy and total nutrient considerations,” it says. This is important considering that FSSAI in its affidavit to the court said that the warning labels would be triggered based on the levels of added sugars and added fats in the food items.

The institute also said that

the warnings should be triggered by a single nutrient as well because it “may result in products substantially high in a single nutrient escaping consumer attention”, the letter says. The FSSAI has proposed that the warnings be triggered only when a food is high in two of the three nutrients in the first phase of implementation, followed by each nutrient in the second phase.

Three, NIN has proposed using higher thresholds for a phased manner of implementation. Its letter states: “If phased implementation is considered necessary for operational reasons, a time-bound transition with higher thresholds than recommended by us, which can be progressively lowered in phases to align with the thresholds, may also be considered, allowing adequate opportunity for product reformulation while avoiding indefinite postponement.”

The Supreme Court has asked FSSAI for a scientifically justified and clearly defined timeline for the implementation of the second phase, without which, the court said, the implementation may take a back seat or be indefinitely postponed. It also sought clarity on which sweetened beverages will receive the warning in the first phase.

FSSAI's front-of-pack warning-label proposal

Non-communicable diseases

Informed consumer choice

ICMR-NIN Dietary Guidelines 2024

India is moving towards a more visible, interpretive food-labelling regime, with the latest debate favouring warnings for individual nutrient excesses rather than requiring multiple nutrient thresholds to be crossed.



AMEER SHAHUL

Our healthcare boom hides a public-system deficit

THE MOST revealing number in India's healthcare story after 12 years of BJP rule may not be the number of hospitals built, Ayushman cards issued or medical seats created. It may be 1.43 per cent. That was government health expenditure as a share of GDP in 2022–23 against the 2.5 per cent target set by the National Health Policy 2017.

India has expanded almost every visible measure of healthcare. Medical colleges and seats have multiplied. The footprint of AIIMS has expanded. Ayushman Bharat has become one of the world's largest publicly funded health insurance programmes. Yet the latest report of the Parliamentary Standing Committee on Health and Family Welfare finds a system still struggling with basic capacity, affordability and regulation.

Consider medical education. The number of medical colleges has risen to 818 and undergraduate medical seats have reached 1,28,875, an extraordinary expansion. But India's problem was never simply the number of doctors. It was where they practise, who can afford them and whether the system has the infrastructure to use them. The Committee finds specialist shortages at rural community health centres of roughly 70–80 per cent. There are 17,788 building-less sub-centres. So India has produced many more doctors without solving the problem of distributing healthcare capacity.

Medical education has also become a major commercial sector,

But India's problem was never simply the number of doctors.

It was where they practise, who can afford them and whether the health system has the infrastructure to use them

with private and PPP models playing an important role. When medical seats become valuable commercial assets, the integrity of regulation becomes crucial. This raises a question: Who regulates the regulator?

The same tension runs through healthcare more broadly. The Committee, citing the NSSO 80th round (2025), says more than 60 per cent of hospitalisations and around 70 per cent of outpatient care are serviced by the private sector. Average hospitalisation expenditure is approximately Rs 6,631 in government hospitals against Rs 50,508 in private hospitals. The state does not have to charge a patient Rs 50,000 to impose a Rs 50,000 burden. It only has to fail to provide a realistic public alternative.

This is where the healthcare architecture takes another turn: Financial protection through insurance. Ayushman Bharat has helped millions of families obtain hospital care they might otherwise have been unable to afford. But insurance can pay the bill without controlling its price. If treatment costs Rs 5 lakh and insurance pays Rs 5 lakh, the patient may be protected from immediate financial ruin. But the healthcare system has still consumed Rs 5 lakh. If prices continue rising, somebody eventually pays — government, insurer, employer or patient.

Meanwhile, more than 40 crore Indians — the missing middle — remain outside comprehensive financial protection. Out-of-pocket

expenditure, although it has fallen substantially, still accounted for 43.4 per cent of total health expenditure in 2022–23.

As healthcare becomes more expensive and the public system fails to keep pace with demand, private capital enters to fill the gap. Major transactions involving businesses such as Manipal Health Enterprises, CARE Hospitals, KIMS, Rainbow Children's Hospital and several diagnostic chains illustrate the growing appetite for healthcare assets. Investors are attracted by recurring demand, consolidation opportunities and the ability to build scalable chains. Capital is not inherently bad for healthcare. India needs enormous investment. The question is what that investment produces. Does it create capacity in under-served districts? Does it make treatment cheaper? Does it strengthen primary care? Or does it acquire existing businesses, consolidate markets and pursue the most profitable segments of healthcare?

The 2026 Lancet Commission Report on a Citizen-Centred Health System for India argues for a stronger publicly financed and publicly provided health system as the foundation of universal healthcare. That should be the ultimate test — whether a family is less vulnerable when illness strikes. If the choice remains between a government hospital that may lack capacity and a private hospital it may struggle to afford, then something fundamental remains unfinished.

Shahul's latest book is *The Silent Syndicate: How Big Finance is Destroying India's Healthcare*

National health policy 2017 target – 2.5% of GDP (1.43%)

Out of Pocket expenditure

Financial protection; Insurance-Ayushman Bharath

“Public health system has limitations in providing universal health coverage. Do you think that the private sector can help in bridging the gap? What other viable alternatives would you suggest?”

— UPSC CSE Mains 2015, GS-II

Strengthen **Health and Wellness Centres/Ayushman Arogya Mandirs**.

- Increase public-health expenditure and improve utilisation of existing resources.
- Address shortages of doctors, nurses and allied health professionals in underserved regions.
- Create incentives for specialists to serve rural and difficult areas.
- Strengthen district hospitals rather than concentrating resources only in tertiary centres.
- Improve regulation of private healthcare and transparency in treatment costs.
- Expand insurance coverage while reducing exclusions and improving portability.
- Integrate **preventive, promotive, curative and palliative care**.
- Use telemedicine and digital health to bridge geographical gaps.

Thank you

“The price of success is daily effort”



gallantias@gmail.com



+91 9746035110